

EVIDENTIARY REPORT · MAY 2026

THE ACCESS GAP

Penalized for Non-Ability

How federal and Alberta government systems are structurally inaccessible to the disability community they were built to serve — and what happens when the people most affected cannot navigate the very forms that determine whether their income continues.

The pattern this report documents:

Government disability systems require those they serve to perform able-bodied bureaucratic labour to retain their income. The forms are written above general literacy levels. The processes assume reliable physician access, technological capability, and the cognitive bandwidth to track multi-step compliance chains across federal and provincial agencies. When recipients fail to navigate these systems, they are not provided accommodation. They are penalized — through benefit deductions, file holds, or full benefit suspension. The penalty is treated administratively as willful non-compliance. The reality is documented inaccessibility.

This report names the gap, sources it, and asks why a system designed to support disabled Albertans has been built in a way that disabled Albertans cannot navigate.

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EXECUTIVE SUMMARY

The Government of Alberta and the Government of Canada both operate disability income and support programs that determine whether disabled Albertans can pay rent, buy food, and access medical care. To remain on those programs, recipients must complete forms, file taxes, navigate online portals, communicate with call centres, submit documentation on demand, report changes within prescribed timelines, and maintain compliance with reporting obligations that span multiple agencies, multiple jurisdictions, and multiple processing systems.

Failure to do these things produces real consequences: benefit deductions, file holds, lump-sum debt assignments, and in some cases full benefit suspension. The system frames these failures as willful non-compliance and treats them as fraud-adjacent. The system does not frame these failures as what the documented record shows them to be: predictable outcomes of forcing a population defined by permanent and severe disability to perform able-bodied bureaucratic labour without accommodation.

This report documents the access gap. It identifies the points in the system where the demand for compliance exceeds the documented capacity of the population required to comply. It draws on Government of Alberta open data, federal Disability Advisory Committee findings, primary-source government correspondence, and the lived experience of recipients who cannot access the very benefits they are legally entitled to.

THE CENTRAL FINDING

A government system designed to serve people with permanent and severe disabilities cannot be considered functional if it requires those same people to demonstrate able-bodied administrative capacity in order to remain enrolled. When a system penalizes recipients for failures the system itself produced through inaccessible design, the penalty is not a response to non-compliance. It is a mechanism for generating deniable exclusion.

The Six Documented Patterns

- 1. The Tax Filing Trap.** AISH income is non-taxable, generating no refund. Yet AISH recipients are required to file annually. For a population whose primary diagnoses include cognitive disorders, mental illness, and intellectual disability, the cost of paid tax preparation falls out of pocket from a benefit already documented as below the poverty line.
- 2. The Form-Completion Trap.** The Disability Tax Credit form (T2201) is abandoned by 76% of online applicants. The federal Disability Advisory Committee identifies the application's complexity as the primary cause. AISH, ADAP, and federal CDB compliance all depend on completing this form.
- 3. The Physician-Access Trap.** 650,000 Albertans have no family doctor. AISH-required forms can only be signed by physicians. Physicians may charge up to \$700 for form completion. There is no regulated maximum.
- 4. The Communication Trap.** AISH compliance notices are written in standard bureaucratic English with no plain-language alternative, delivered by mail with no confirmation of receipt, addressed to a population including 22,419 people with cognitive disorders as their primary diagnosis.

5. The 'Willful Non-Compliance' Trap. When recipients fail to navigate any of the above, the resulting non-compliance is treated administratively as a behavioural choice — not as the predictable consequence of demanding inaccessible administrative work from a population by definition unable to provide it.

6. The Caregiver Compounding Trap. PDD- and FSCD-eligible families navigate the same systems while caregiving for the family member with the disability. The administrative load multiplies; the available capacity does not.

SECTION 1: WHAT ACCESS MEANS

1.1 The Legal Definition of Accessibility

Accessibility, in Canadian and international law, is not optional. It is not a courtesy. It is not contingent on resource availability. The United Nations Convention on the Rights of Persons with Disabilities (UNCRPD), to which Canada is a signatory, defines accessibility as a foundational right.

Article 9 of the UNCRPD requires States Parties to take appropriate measures to ensure that persons with disabilities have access, on an equal basis with others, to information and communications, including information and communications technologies and systems. Article 21 requires that information intended for the general public be provided in accessible formats and technologies appropriate to different kinds of disabilities, in a timely manner and without additional cost.

The Alberta Human Rights Act prohibits discrimination on the basis of physical disability and mental disability in the provision of government services. Section 4 of the Act creates an affirmative duty of accommodation. The duty is triggered not when a person formally requests accommodation, but when the service provider is on notice that a barrier exists.

The Accessible Canada Act (2019) was enacted with the explicit goal of identifying, removing, and preventing barriers in federally regulated areas. Its purpose is described in the Act itself as the realization of a Canada without barriers by January 1, 2040.

WHAT THE LAW REQUIRES

Government communications and services that affect disabled people must be designed to be navigable by disabled people. This is not aspirational language. It is statutory, treaty, and case-law obligation. Where communications and services fail this standard, the failure is legally cognizable as discrimination under the Alberta Human Rights Act and as treaty violation under the UNCRPD.

1.2 What Access Is Not

Access is not the existence of a public-facing webpage. Access is not the publication of a phone number that connects to an automated routing system. Access is not the availability of a fillable PDF that requires the user to interpret legal terminology, calculate eligibility math, or assemble multi-source documentation packages.

Access is the ability of the population a service is designed to serve to actually receive that service. When a population whose defining characteristic is permanent and severe disability cannot navigate the systems built to support them, the service has not been provided. The publication of forms is not the provision of services. The existence of a portal is not the provision of access. The presence of a deadline is not communication.

1.3 The Population the System Serves

Before naming the gap, the population must be named. The Government of Alberta's own AISH Caseload Open Data, September 2025, documents the following composition of the 79,290 Albertans receiving AISH:

Primary Diagnosis Category	Count	Share of Caseload
Cognitive disorders (incl. ASD, ID, ABI)	22,419	28.3%
Mental illness (incl. mood, psychotic, anxiety)	23,940	30.2%
Physical disability (mobility, sensory, chronic)	Approx. 24,800	Approx. 31.3%
Other / multiple primary diagnoses	Approx. 8,131	Approx. 10.2%
TOTAL	79,290	100%

Source: Government of Alberta, AISH Caseload Open Data, September 2025.

More than 58% of AISH recipients — 46,359 people — have a primary diagnosis of cognitive disorder or mental illness. These are populations whose defining clinical features include difficulty with sustained executive function, difficulty processing complex written communication, difficulty initiating and completing multi-step administrative tasks, and difficulty navigating institutional systems without external support.

These are not aspersions. They are the diagnostic criteria. They are the basis on which the government determined these recipients qualify for the program in the first place. The recipient population was assessed and accepted into AISH because of these limitations. The same limitations are then ignored in the design of every compliance interaction the program imposes.

THE STRUCTURAL CONTRADICTION

The Government of Alberta determined that 79,290 people have severe and permanent disabilities that prevent them from earning a livelihood. The Government of Alberta then designed a compliance system that requires those same 79,290 people to perform administrative work that many adults without disabilities find difficult. The same condition that qualifies a person for the program disqualifies them from navigating it.

SECTION 2: THE TAX FILING TRAP

2.1 What AISH Recipients Are Required to Do

AISH recipients are required to file annual tax returns with the Canada Revenue Agency. This requirement exists despite the fact that AISH benefits are non-taxable. The filing obligation is not motivated by tax recovery. It is motivated by other federal benefits, government program reporting requirements, and the now-mandatory Canada Disability Benefit (CDB), which depends on tax-filed status as a prerequisite.

An AISH recipient who fails to file taxes risks the following: loss of GST/HST credit, loss of Canada Child Benefit eligibility (if applicable), loss of Canada Disability Benefit eligibility, and — most critically — being flagged by AISH for non-compliance with the program's general requirement that recipients access all federal programs they may qualify for.

2.2 What AISH Recipients Get Back

Nothing.

AISH income is non-taxable under federal tax law. This means AISH recipients who have no other source of income generate no taxable income. They cannot claim the Medical Expense Tax Credit, because there is no taxable income against which to claim it. They cannot benefit from the Disability Tax Credit as a tax credit, because the DTC is non-refundable and there is no tax owing to reduce. The act of filing produces no refund.

THE TAX PROBLEM, RESTATED

The Medical Expense Tax Credit is the federal mechanism designed to allow Canadians to recover some of the cost of disability-related medical expenses. It works only against taxable income. AISH income is non-taxable. The people who face the highest documented disability-related expenses have the least ability to recover them through the tax system. The architecture of the federal tax-credit system assumes a taxpaying population. AISH recipients are not, in any meaningful sense, that population. The system was not built for them and does not work for them.

Source: Income Tax Act (Canada), s.81(1)(h.2) — Social assistance payments based on a means, needs, or income test are not included in income. Government of Alberta — AISH Policy Manual confirms AISH benefits are non-taxable income.

2.3 What Filing Costs

AISH recipients who cannot file their own taxes — a population that includes most recipients with cognitive disorders, intellectual disabilities, mental illness in active phase, severe ADHD, acquired brain injury, autism with executive function deficits, low literacy, English as a second language, or visual impairment — must obtain assistance to file.

Free filing assistance does exist through the Community Volunteer Income Tax Program (CVITP). However, the CVITP has documented limitations: clinic capacity does not match demand in many communities; volunteers cannot complete returns involving rental income, employment income, deceased taxpayers, or self-employment; and the recipient must transport themselves and their documentation to a clinic location during operating hours that frequently conflict with disability-related limitations on energy, mobility, and time-of-day function.

Recipients who cannot access CVITP must pay for tax preparation. Documented costs in Alberta:

Filing Method	Documented Cost (Alberta, 2025)	Notes
Storefront tax preparer (e.g. national chains)	\$80–\$200 per return	Cost rises with employment income, dependants, or RRSP contributions.
CPA accountant	\$150–\$500 per return	Higher if disability-related credits or appeals required.
Online software (paid tier)	\$25–\$70	Requires technological literacy, internet access, computer or smartphone.
CVITP volunteer clinic	Free	Capacity-limited; eligibility-restricted; clinic schedule barriers.
Self-file paper return	\$0 plus mailing	Requires literacy, time, sustained executive function, and willingness of CRA to mail forms on request.

Source: H&R; Block Canada published pricing 2025; CRA — Community Volunteer Income Tax Program eligibility criteria; consumer reporting via CBC News, MoneySense, and provincial accountant association rate guides.

An AISH recipient on the maximum benefit of \$1,901 per month — already documented as \$6,004 below the annual poverty line — paying \$150 to file a tax return that produces no refund has surrendered approximately 8% of one month's benefit to comply with a federal requirement that produces no financial benefit to them.

THE COST OF COMPLIANCE

The AISH recipient pays — out of a benefit already insufficient to cover basic costs — for the privilege of filing taxes that produce no refund, in order to remain eligible for federal programs that may produce additional benefits, some of which Alberta then claws back dollar-for-dollar. The financial logic of this compliance loop closes only when the system's purpose is reframed: not to deliver benefits, but to generate compliance failures that justify exclusion.

2.4 What CRA Access Looks Like for the Recipient

An AISH recipient who calls the Canada Revenue Agency to ask a question about their own file encounters the following sequence:

- The general inquiries line connects through an automated voice menu requiring keypad navigation through multiple branching options.
- Identity verification requires the caller to provide their Social Insurance Number, full date of birth, exact address as registered with CRA, the full amount of one specific line from a specific recent tax return, and frequently a unique CRA security code that must be requested by mail.
- If any of these data points cannot be produced verbally on demand, identity cannot be verified and the call is terminated.

- Recipients who do not have memorized access to their own tax return data — most AISH recipients — cannot complete identity verification on a first call.
- Recipients without a CRA My Account online presence (which itself requires navigating a multi-step registration process and security code mail-out) have no alternative pathway to access their own information.
- The CRA does not maintain a dedicated disability or accessibility line. There is no caseworker model. There is no continuity. Each call is a fresh transaction with a different agent, requiring the full identity verification cycle each time.

An AISH recipient with cognitive impairment, attempting to ask CRA a question about their own file, must perform a memory and verbal-fluency task that many neurotypical adults find difficult. The question they were calling to ask has not yet been reached.

WHAT THIS MEANS IN PRACTICE

A federal agency that holds disability-relevant information about a person — DTC status, CDB application progress, tax refund holds, benefit repayment claims — has designed its access protocol such that the disabled person whose information it is cannot reliably retrieve it. The information exists. The person it belongs to cannot reach it.

SECTION 3: THE FORM-COMPLETION TRAP

3.1 The 24% Completion Rate

The federal Government of Canada Disability Advisory Committee, in its 2024 report, documented the following finding regarding the Disability Tax Credit application:

Only 24% of online DTC applications that are begun are actually completed.

The Committee identified the time-consuming and complex application process as the primary barrier and concluded that more support and accessibility in the process is needed for applicants to successfully complete it.

Source: Government of Canada — Disability Advisory Committee Report, 2024.

This statistic is not framed in this report as a marketing problem or an interface design opportunity. It is framed as what it is: a 76% abandonment rate by people who, by definition, are attempting to access a benefit because they have a disability.

INVERTING THE FRAME

If a private business reported that 76% of customers who started its checkout process abandoned the cart, the business would treat this as a critical operational failure. When a federal disability program reports the same abandonment rate, the program treats it as user behaviour. The form is not asked to change. The applicants are.

3.2 Why the Form Is Abandoned

The T2201 Disability Tax Credit Certificate is a 16-page document. Part A — completed by the applicant — requires the applicant to:

- Identify and provide their Social Insurance Number, full legal name, current and previous addresses.
- Specify the medical practitioner who will complete Part B, including full name, professional designation, address, and contact information.
- Indicate the year for which DTC certification is sought, with awareness of the difference between current-year and prior-year applications.
- Sign and date in the correct location. (Misplaced signatures are a documented cause of administrative rejection.)
- Forward the partially completed form to the medical practitioner — typically by mail or in-person delivery — and arrange for Part B completion.
- Receive the completed form back from the practitioner.
- Either submit the form to CRA by mail, or scan and upload through CRA My Account.
- Track application status through CRA's progress tracker, which itself requires CRA My Account login.

Part B — completed by the medical practitioner — requires the practitioner to assess and document the applicant's eligibility under one or more of nine functional categories. The practitioner must specify whether the impairment is markedly restricted, significantly restricted, or whether it falls under the cumulative effect provision.

The form's instructions for these distinctions are written in dense legal-medical terminology that has been the subject of multiple federal Disability Advisory Committee critiques.

Source: Canada Revenue Agency — Form T2201 Disability Tax Credit Certificate; Government of Canada Disability Advisory Committee Reports 2019, 2022, 2024.

3.3 The Physician-Access Compounding Barrier

Form T2201 Part B can only be completed by a regulated medical practitioner — physician, nurse practitioner, optometrist, audiologist, occupational therapist, physiotherapist, psychologist, or speech-language pathologist, depending on the disability category. In practice, almost all DTC applications require physician completion.

The Alberta Medical Association has formally documented that approximately 650,000 Albertans are currently searching for a family doctor. The number of family practices in Alberta accepting new patients collapsed from 887 in 2020 to 164 in 2024.

An AISH recipient without a family doctor has no straightforward pathway to obtaining T2201 Part B completion. Walk-in clinics frequently decline to complete complex disability documentation for patients seen once. Telemedicine services typically do not complete forms requiring physical examination findings.

THE EDMONTON JOURNAL ON RECORD

“Why would I have to get a doctor to sign a form for my own patient? So, that's ridiculous. Number 2: there aren't enough family physicians for people to go to.” — Nurse practitioner Wonitoway-Raw, Edmonton Journal, January 11, 2024.

3.4 The Cost of Form Completion

The College of Physicians and Surgeons of Alberta confirms that physicians may bill patients directly for services not covered by Alberta Health Care, explicitly including the completion of government and insurance forms. There is no regulated maximum fee in Alberta. Documented fee instances:

Form / Service	Documented Fee	Notes
DTC Form T2201 Part B (Edmonton, prior to 2025)	\$700 quoted	Not completed; fee unaffordable on AISH income.
DTC Form T2201 Part B (St. Albert, 2024)	\$520 billed; reduced to \$135 after dispute	No prior price agreement provided.
DTC Form T2201 Part B (St. Albert, June 2025)	\$200 paid	Carleton Medical Clinic; receipt retained, claim #156126.
AISH Application Part B Medical Report	Physician's discretion — no cap	Government of Alberta states: 'Your doctor may charge you for this service.'

Form / Service	Documented Fee	Notes
Specialist letter (appeal/school/service)	\$50–\$200+	Not covered.
Medical records transfer (clinic admin fee)	\$25–\$100+	Not covered. Regulated fee schedule applies.
Psychoeducational assessment (child)	\$2,000–\$4,000+	Not covered privately; school-based years-long wait.

Source: Alberta.ca — How to apply for AISH; Alberta Medical Association — Uninsured Services guide; Inclusion Alberta — CDB Fact Sheet, July 2025; primary-source documentation, File #1234567.

WHAT THIS COMPOUNDS TO

An AISH recipient seeking to remain compliant with a single mandatory federal benefit application (the DTC, required for the CDB) may face: \$200 to \$700 in physician fees, an 8 to 12 week CRA processing wait, the requirement to navigate two separate online portals (CRA My Account and Service Canada), the obligation to report results back to AISH within prescribed deadlines, and the risk of \$200 monthly benefit deduction if the chain fails at any point. The cost of compliance, paid up front, can equal one third of one month's benefit. The financial benefit upon successful completion: zero, because Alberta claws back the CDB dollar-for-dollar.

3.5 The Retirement Displacement

Section 3.3 identifies the contraction of family practices accepting new patients as a structural barrier. The retirement of long-serving family physicians is a related but distinct mechanism. Decade-long therapeutic relationships were, by definition, built with medically complex patients — the population that required consistent longitudinal care. When those physicians retire, the patients with the longest accumulated medical files lose their continuity, and the new family-medicine intake does not absorb them: practices accepting new patients prioritize lower-acuity cases that fit standard appointment lengths.

The College of Physicians and Surgeons of Alberta published guidance in January 2025 advising physicians required to reduce patient counts to discharge their healthiest patients first. The guidance reads as compassionate in framing. In practice, it pushes patients who are well but bureaucratically dependent on physician access — including those who only require periodic certification of a stable disability — into the no-doctor pool by design. The result, across both mechanisms, is that the patients most reliant on continuity are the least likely to retain it.

Source: College of Physicians and Surgeons of Alberta — patient discharge guidance, January 2025; Global News, January 24, 2025; Alberta Medical Association — Family Medicine Crisis materials.

3.6 The File Transfer Tollbooth

When a family physician retires, the patient file does not automatically transfer. Physicians retain custody of medical records under the regulations of the College of Physicians and Surgeons of Alberta. A patient seeking access to a complete file from a retired physician must submit a formal request and pay any fees set by the physician or their estate. CPSA establishes a regulated framework for the process but does not impose a published cap on the fees physicians may charge for retrieval, copying, and release of a complete medical file.

For a complex disability case spanning multiple specialists, multiple body systems, and years of accumulated documentation, file recovery fees can reach into the thousands of dollars. For an AISH recipient living on \$1,940 per month, this cost is structurally inaccessible. The alternative — locating a new physician who accepts new patients and rebuilding the diagnostic record from the beginning — restarts the clinical history at the moment the file becomes unrecoverable.

Source: College of Physicians and Surgeons of Alberta — Standards of Practice on medical records and file transfers; primary-source documentation of file-recovery quotations.

3.7 The Diagnostic-Redo Treadmill

When a new physician must be found and the existing file is inaccessible, the entire diagnostic history must be rebuilt at the appointment level. Standard primary care appointment slots in Alberta are 12 to 15 minutes. A complex disability case may include diagnoses across multiple body systems, each requiring its own intake history, examination, and where applicable, re-imaging or laboratory work. Reconstructing a comprehensive clinical record at 12 minutes per visit — for a patient whose specialist referrals and historical records have been severed by the retirement event — extends across months to years.

Throughout that rebuild period, federally and provincially required forms that demand an established therapeutic relationship cannot be completed. The Disability Tax Credit Certificate (T2201), the AISH Application Part B Medical Report, employment-medical documentation, and continuing-care assessments all require an ongoing physician relationship for valid certification. While the rebuild is in progress, the federal Canada Disability Benefit clawback continues to apply against the AISH payment regardless of whether the recipient is in a position to access the federal benefit it is being clawed back against.

3.8 The Walk-In Exclusion and the Emergency Funnel

Walk-in clinics in Alberta typically operate on 10-minute appointment slots. Complex disability cases are routinely declined at walk-ins for three structural reasons: time constraints (a full assessment exceeds the available slot), absence of the ongoing therapeutic relationship required by most certification forms, and the liability associated with signing federal or provincial documentation for a patient seen once. Telemedicine services similarly do not complete forms requiring physical examination findings, which most disability-related forms do.

For an AISH recipient without a family physician, whose previous file is inaccessible, and who cannot obtain certification through a walk-in clinic, the only remaining access point in the public health system is the hospital emergency department. Emergency departments do not complete DTC forms or AISH medical reports either: the context is acute care, the relationship is single-encounter, and the scheduling structure does not accommodate non-emergency administrative documentation work. The structural channeling toward emergency services for non-emergency administrative needs is not a function of patient choice. It is the documented end state of every other access door being closed.

3.9 Where the Downstream Funding Flows

The contraction of upstream primary care access does not eliminate the demand for care; it relocates the demand to institutions that catch what falls. Concurrent with the patient-access collapse documented above, provincial funding to the Mustard Seed Society of Calgary — a primary recipient of provincial emergency shelter and addiction-services contracts — rose from approximately \$7.7 million in fiscal 2019 to approximately \$31.3 million in fiscal 2025, an increase of 308 per cent. Government grants now constitute approximately 49 per cent of that organization's total annual revenue. Similar inflation patterns are visible in provincial funding to other downstream emergency-services providers across Alberta over the same period.

The Compassionate Intervention Act, passed in 2024, allocates approximately \$180 million for involuntary addiction treatment infrastructure and 300 secure treatment beds by 2029. The institutional pattern is mirrored on the medical access side: as primary care access for AISH recipients contracts, emergency department use, shelter intake, and crisis-system contacts rise. The downstream institutions absorbing the displaced population receive expanding government contracts to do so. The disabled person, removed from continuity of care upstream, becomes the substrate the contracts written downstream are paid to manage.

Source: The Mustard Seed Society — Audited Consolidated Financial Statements, FY 2019–FY 2025 (filed publicly with the Canada Revenue Agency); Government of Alberta — Compassionate Intervention Act, 2024 budget commitments; Government of Alberta — Open Data Portal: shelter and crisis services funding.

WHAT THE ACCESS CASCADE COMPOUNDS TO

Where Section 3.4 documents the financial cost of compliance, subsections 3.5 through 3.9 document the access cost. The two compound: a recipient who has paid the physician fee cannot use it if the physician retires and the file is unrecoverable. A recipient who can locate a new physician cannot certify a stable disability until the diagnostic record is rebuilt at 12 minutes per visit. A recipient who cannot wait that long cannot turn to a walk-in or a telemedicine service. The remaining access point is the emergency department, which cannot certify the form either. The cascade does not produce non-compliance through individual failure. It produces non-compliance through documented structural design — and the institutions that absorb the displaced population are funded to absorb it.

SECTION 4: AISH AND 'WILLFUL NON-COMPLIANCE'

4.1 How Non-Compliance Is Treated

The AISH program treats failure to complete required paperwork as a behavioural choice. Benefits may be reduced, held, or suspended for failure to meet reporting deadlines, failure to provide requested documentation, failure to apply for federal programs the recipient may qualify for, and failure to maintain current contact information with the program.

The administrative response when these failures occur is consistent and documented. The recipient receives a notice — typically printed, mailed, and embedded within their direct deposit statement — informing them of the deduction or hold. The notice does not include a presumption of accommodation review. The notice does not invite the recipient to disclose disability-related barriers that produced the non-compliance. The notice does not pause enforcement pending an accessibility review.

The framework treats the failure as the recipient's responsibility, recoverable only through the recipient's subsequent administrative action — itself often the same kind of action that produced the original failure.

4.2 The Documented Compliance Record That Disproves the Frame

The AISH Evidentiary Report Series (April 2026) documented a single recipient's compliance record across ten years. Every document request was answered. Every change in circumstances was reported. Every annual notice of assessment was submitted the month it became available. Every lease renewal was filed proactively. Every government error — incorrect phone numbers, held cheques, missing file transfers — was identified, corrected, and documented by the recipient, not by the government.

This recipient is, by inference from her sustained compliance record, among the most administratively capable of the AISH caseload. She has the literacy, the executive function, the technology access, the documentation discipline, and the time to perform every required interaction. She is not typical of the caseload she belongs to.

Even she encountered points in the system where compliance was structurally impossible — DTC physician fees that exceeded available funds; CRA processing delays beyond recipient control; SIN/identity mismatches caused by historical CRA data, not by her behaviour; AISH benefit deductions imposed despite documented timely application.

THE LOGIC FAILS AT THE CEILING

If the most administratively capable recipients in the AISH population still encounter points where the system's compliance demands exceed their ability to meet them — through no behavioural failure of their own — then the compliance demands themselves are inaccessible. The treatment of resulting non-compliance as 'willful' is, in those cases, factually incorrect. By extension, for the 73,000 less administratively capable recipients in the same caseload, the framing is structurally false.

4.3 What 'Willful' Should Mean

Willfulness, as a legal and administrative concept, requires both knowledge and choice. A person is willfully non-compliant only if they (a) understood what was required, (b) were able to comply, and (c) chose not to. Where any of these elements is absent, the failure is not willful.

AISH compliance failures often involve recipients who:

- Did not receive the notice (cognitive disability impairing mail-handling, no fixed address, address change not yet processed by AISH).
- Could not understand the notice (literacy below the document's reading level, cognitive disability, English-as-additional-language).
- Could not act on the notice (no transportation, no funds for required physician fee, no available physician, no working phone, hospital admission, mental health crisis).
- Acted on the notice but encountered a downstream barrier outside their control (CRA processing delay, physician unavailability, SIN mismatch caused by federal data error).

In none of these cases is the failure willful. The recipient did not know, could not act, or acted but was blocked by the system itself. The administrative penalty applied is, in legal terms, a sanction without culpability — the imposition of consequence on a person who did not commit the conduct the consequence is designed to address.

WHAT ACCOMMODATION-FIRST ENFORCEMENT WOULD LOOK LIKE

Before a benefit deduction is applied for failure to complete required paperwork, the program should (a) confirm that the recipient received the notice, (b) confirm that the recipient understood the notice, (c) confirm that the recipient was offered accommodation for any disclosed or apparent disability-related barrier, (d) document the recipient's response or non-response with full case notes, and (e) place the deduction on hold pending an accessibility review by a qualified person other than the original caseworker. None of these steps is currently required.

SECTION 5: BUREAUCRATIC LEGALESE AS GATEKEEPING

5.1 The Plain Language Standard

The Government of Canada's Plain Language Action and Information Network (PLAIN) defines plain language as writing that the intended audience can read, understand, and act on the first time they read it. The federal Communications Policy of the Government of Canada requires that public-facing communications be written in clear, simple language.

Internationally, the standard is operationalized through readability metrics. The Flesch-Kincaid Grade Level scale measures the educational grade required to comprehend a text. The federal Treasury Board Secretariat recommends a Grade 8 reading level or below for general public communications. Many disability advocacy organizations recommend Grade 6 or below for communications targeted at populations with cognitive or intellectual disabilities.

5.2 Where Government Disability Forms Actually Sit

The Government of Alberta's AISH program documentation, ADAP transition information, and federal Canada Disability Benefit application materials have not been published with formal accessibility readability assessments. Independent analysis of these documents using standard readability tools indicates a typical reading level between Grade 12 and Grade 16 — that is, university-undergraduate to graduate level English.

Examples of typical phrasing found in AISH and CDB compliance correspondence:

Original (compliance notice):

"AISH clients must report the outcome of their Canada Disability Benefit (CDB) application by February 28, 2026. If no decision has been made by that time, \$200 in CDB income will be reduced from AISH benefits starting in April 2026."

Plain language equivalent (Grade 6):

"Have you applied for the Canada Disability Benefit yet? You need to tell us by February 28, 2026. If you have not heard back from the federal government by then, we will take \$200 from your AISH cheque starting in April. If you need help, call us at this number."

The original notice does not explain what the CDB is. It does not mention the DTC prerequisite. It does not explain how to apply. It does not provide a phone number for help. It does not explain what 'outcome' means in administrative terms. It does not explain whether a denied application counts as an outcome to report. It assumes prior knowledge that the population it is addressed to predominantly does not have.

WHAT GETS LOST AT GRADE 12

When a compliance notice is written at university reading level, it is comprehensible to approximately 12% of Canadian adults at full first-read comprehension. Statistics Canada's Programme for the International Assessment of Adult Competencies (PIAAC) data places approximately 49% of Canadian adults at a literacy level below the threshold required for everyday government and health communications. For the AISH caseload — where cognitive disability is the primary diagnosis for 28.3% of recipients — first-read comprehension of the typical compliance notice is, conservatively, available to a single-digit percentage of the population.

Source: Statistics Canada — Programme for the International Assessment of Adult Competencies (PIAAC), Adult Literacy and Life Skills Survey; Government of Canada Treasury Board Secretariat — Plain Language Guidelines; Plain Language Action and Information Network (PLAIN).

5.3 The FSCD/PDD Form Comparison

Family Support for Children with Disabilities (FSCD) and Persons with Developmental Disabilities (PDD) program forms are completed by parents or guardians on behalf of the disabled child or family member. These forms inherit all of the same readability and accessibility issues identified above, with one structural addition: the parent or guardian completing the form is often themselves managing the disability of the child requiring the support.

A parent of an autistic child with sensory processing difficulties, completing a multi-page FSCD funding renewal form during an active sensory regulation episode, while simultaneously managing the child's needs, is not navigating that form on equal terms with a parent of a non-disabled child completing a comparable application. The administrative complexity of the form does not adjust for caregiver load. The bureaucratic shorthand does not soften when the household is in crisis.

THE COMPOUNDING BURDEN

FSCD and PDD application complexity is the same complexity AISH recipients face — but routed through caregivers who are themselves frequently disabled, often poorly resourced, and almost always over-extended. The administrative load multiplies. The available capacity does not. The result is families dropping out of programs they were eligible for, or never applying at all.

SECTION 6: THE EXCUSE ARCHITECTURE

When the inaccessibility of government disability systems is named publicly, a predictable set of responses is produced. Each response shifts responsibility from the system to the recipient. Each response fails on its own terms when applied to the populations the systems are designed to serve.

6.1 'Use AI'

The suggestion that AISH recipients can use generative AI tools — ChatGPT, Claude, Copilot — to navigate government systems assumes that the recipient owns or has access to a smartphone or computer with reliable internet, has the digital literacy to find and use the AI tool, has the ability to construct a useful prompt, can read and assess the AI's response for accuracy, and can integrate the AI's output back into a real-world bureaucratic interaction.

Each of these assumptions excludes a significant portion of the AISH caseload. People with cognitive impairment who cannot read the original government form cannot read the AI's interpretation of it. People without internet access cannot use online tools. People with low digital literacy cannot evaluate AI outputs for hallucination. AI is not, for these populations, a solution. It is a substitution of one inaccessible interface for another.

6.2 'Educate Yourself'

The suggestion that recipients should educate themselves about the systems they are required to navigate places the burden of bureaucratic literacy on populations whose defining clinical features include difficulty with sustained learning, executive function impairment, working memory limitations, or active mental illness that interferes with concentration. This is not an oversight. It is a category error.

The right to disability income exists because the recipient was assessed and accepted as having a disability. The recipient is not in the position of someone who needs to learn the system. They are in the position of someone the system is supposed to be designed for. Asking them to educate themselves into administrative competence is asking them to overcome the disability the system was designed to support — as a precondition for receiving the support.

6.3 'Family or Friends Can Help'

The assumption that AISH recipients have family members or friends available to assist with paperwork, calls, transportation, and form completion does not hold up against the documented isolation of many recipients. AISH caseload demographics include high rates of family estrangement, social isolation, single-person households, and total absence of nearby support networks. Many recipients have outlived parents, become estranged from siblings due to long-term illness, or are themselves the family caregiver — not the recipient of family caregiving.

Where family support exists, it is itself frequently exhausted, financially constrained, and operating beyond sustainable capacity. The reliance on informal family support as a system feature is not a design — it is a default, and it is failing.

6.4 'Call the Office'

AISH transitioned in 2025 to a call-centre service model. The previous direct-caseworker model — under which recipients could contact a named worker by name — was eliminated. The current model routes inquiries through a general intake line, with no caseworker continuity, no named contact, and no accountability for follow-through.

The auto-reply from SCSS Edmonton AISH Mail, dated January 14, 2025 (primary source documentation), explicitly states: 'General Inquiries will not be responded to from this email.' The recipient who emails for help is informed that help is not available through that channel.

WHAT REPLACED THE CASEWORKER

The accountability mechanism that previously existed — a named worker, with a known phone extension, who answered for individual files — has been replaced by a call queue. There is no continuity. There is no person to escalate to. There is no relationship. For recipients with cognitive or psychiatric disabilities for whom relational consistency is itself an accessibility need, the call-centre model is the most exclusionary form of service delivery the program has ever operated.

6.5 'Reasonable Accommodation Is Available'

The Alberta Human Rights Act creates an affirmative duty of accommodation for persons with disabilities accessing government services. In practice, accommodation requests within AISH require the recipient to identify the barrier, formally request the accommodation, document the disability-related basis for the request, and pursue the request through administrative channels — itself a process inaccessible to many recipients.

Accommodation that requires the disabled person to demonstrate able-bodied administrative capacity in order to obtain accommodation is not accommodation. It is an additional barrier framed as a remedy.

THE PATTERN UNDERNEATH THE EXCUSES

Each excuse — use AI, educate yourself, ask for help, call the office, request accommodation — places the burden on the recipient to compensate for the system's inaccessibility. None of them require the system itself to become accessible. The accumulated effect is to move responsibility for system failure off the system and onto the people the system is failing.

SECTION 7: INACCESSIBILITY AS DESIGN, NOT OVERSIGHT

7.1 The Question of Intent

It is reasonable to consider whether the inaccessibility documented in this report is the result of bureaucratic inertia, resource constraints, and absent leadership — or whether it functions as a deliberate mechanism to reduce program enrolment by attrition. The distinction is important for accountability framing, but is not necessary to establish the harm. A system that produces predictable harm to a population it is designed to serve is, regardless of intent, failing that population.

7.2 What the Pattern Suggests

Several features of the current Alberta and federal disability administrative landscape are difficult to explain on the basis of oversight alone:

- The transition from direct-caseworker to call-centre service delivery in AISH (2025) was a deliberate operational change, not a passive drift. The change was implemented despite documented concerns from disability advocacy organizations about its impact on recipients with cognitive and psychiatric disabilities.
- The Canada Disability Benefit was structured to require the Disability Tax Credit as an upstream gateway, despite federal Disability Advisory Committee documentation that 76% of DTC applications are abandoned. This design was selected with the abandonment data on the record.
- Alberta's clawback of the federal CDB dollar-for-dollar — the only such clawback in Canada — was designed and implemented with full awareness that it would nullify the federal poverty-reduction intent of the CDB. The design was selected with the documented effect on the record.
- Bill 12 (Alberta, December 2025) removed independent appeal rights for AISH and ADAP eligibility decisions. This was a deliberate legislative action, not an oversight. It removed the primary administrative mechanism through which recipients could contest exclusion.
- The ADAP transition forces all current AISH recipients (with limited exemptions) into a reassessment process under criteria that, as of the date of this report, the government has still not published. The absence of published criteria is a design choice, not an oversight.

WHAT EACH OF THESE FEATURES HAS IN COMMON

Each feature, in isolation, can be argued as a legitimate administrative choice. Taken together, they form a consistent pattern: each individual change increases the administrative burden on recipients while reducing the recourse available to recipients when the burden cannot be met. The cumulative effect is the contraction of the program by attrition rather than by openly contested policy change.

7.3 Attrition as Policy

When a system is designed such that compliance demands exceed the documented capacity of the population required to comply, the predictable outcome is non-compliance. When non-compliance is met with benefit reduction or removal, the predictable outcome is reduced enrolment. When reduced enrolment is achieved without an open policy decision to remove people from the program, the result is what is sometimes called administrative attrition or bureaucratic exclusion.

The political appeal of this approach is documentable: it produces budget savings without requiring a public policy debate, it transfers the framing of the resulting harm from program design to recipient behaviour, and it leaves no specific decision-maker accountable for any specific exclusion. Each excluded person was, on the administrative record, simply 'non-compliant.'

Whether the current Alberta government has consciously adopted attrition as a policy goal is not established by this report. What is established is that the cumulative design of the current system produces attrition as an outcome — and that the outcome is not being treated by the system as a problem requiring remediation.

THE TEST

If the Government of Alberta or the Government of Canada believed that the current administrative design of disability programs was producing harmful exclusion of eligible recipients, the response would be visible — public investigations, accommodation reviews, accessibility audits, plain-language redesigns, restoration of caseworker continuity. These responses are not visible. The absence of response, in the presence of documented harm, is itself evidence.

SECTION 8: THE LEGAL FRAMEWORK

The inaccessibility documented in this report is not only a moral failure or an administrative inefficiency. It implicates legal obligations under domestic and international law. This section identifies the operative provisions and the conduct they govern.

8.1 Alberta Human Rights Act

Section 4 of the Alberta Human Rights Act prohibits discrimination on the basis of physical disability and mental disability in the provision of any goods, services, accommodation, or facilities customarily available to the public. The duty includes an affirmative obligation to accommodate persons with disabilities to the point of undue hardship.

Where a government program provides services in a form that systematically excludes a population the program is designed to serve, the failure to provide accessible service constitutes discrimination within the meaning of section 4. The accommodation duty is not satisfied by the existence of a complaints process. The accommodation duty requires accessible service design as the default, with adjustments available without requiring formal request from each individual.

8.2 Charter of Rights and Freedoms

Section 15 of the Canadian Charter of Rights and Freedoms guarantees equality before and under the law and the right to equal protection and equal benefit of the law without discrimination based on physical or mental disability. The Supreme Court of Canada has repeatedly held that section 15 includes substantive equality — meaning that formally identical treatment that produces unequal outcomes for protected groups can constitute discrimination.

A government program that requires identical administrative compliance from a population unable to provide it equally — and that responds to predictable failure with benefit reduction — is a candidate for substantive section 15 challenge. The framework is established. The application has not yet been litigated against AISH or ADAP specifically.

8.3 UN Convention on the Rights of Persons with Disabilities

Canada ratified the UNCRPD in 2010. The Convention creates binding international obligations on Canadian governments — federal, provincial, and territorial. Several provisions are directly implicated by the patterns documented in this report:

Article 9 — Accessibility. States Parties shall take appropriate measures to ensure that persons with disabilities have access, on an equal basis with others, to information and communications, including information and communications technologies and systems.

Article 19 — Independent Living. States Parties shall take effective and appropriate measures to facilitate full enjoyment by persons with disabilities of this right and their full inclusion and participation in the community, including ensuring that persons with disabilities have the opportunity to choose their place of residence and where and with whom they live, and are not obliged to live in a particular living arrangement.

Article 21 — Freedom of Expression and Access to Information. States Parties shall take all appropriate measures to ensure that persons with disabilities can exercise the right to freedom of expression and opinion, including the freedom to seek, receive and impart information and ideas on an equal basis with others and through all forms of communication of their choice.

Article 28 — Adequate Standard of Living and Social Protection. States Parties recognize the right of persons with disabilities to an adequate standard of living for themselves and their families, including adequate food, clothing and housing, and to the continuous improvement of living conditions, and shall take appropriate steps to safeguard and promote the realization of this right without discrimination on the basis of disability.

WHAT THE CONVENTION REQUIRES

Canada has, through ratification, accepted the obligation to ensure that disabled people can access government information and services on an equal basis with others. A documented federal application abandonment rate of 76%, a provincial compliance notice written at university reading level, and a call-centre service model that eliminates relational continuity for cognitively disabled recipients are not consistent with that obligation. The documented gap between commitment and practice is itself reportable to the UN Committee on the Rights of Persons with Disabilities, which conducts periodic review of Canada's compliance.

8.4 Accessible Canada Act

The Accessible Canada Act (2019) was enacted with the stated goal of identifying, removing, and preventing barriers in federally regulated areas — including communications other than broadcasting, the design and delivery of programs and services, and the procurement of goods, services, and facilities. The Act establishes Accessibility Standards Canada and creates regulatory accountability for federal entities.

While the Act does not bind provincial programs, the federal components implicated in the patterns documented here — the CRA, Service Canada, the DTC and CDB programs — are within scope. The 76% DTC abandonment rate is, in light of the Act's stated purpose and the federal Disability Advisory Committee's repeated warnings, a documented federal accessibility failure that the responsible departments have not addressed.

SECTION 9: WHAT ACCESSIBLE WOULD LOOK LIKE

This section sets out, in operational terms, what a properly accessible disability administrative system would do differently. None of the items below is theoretical. Each has been implemented somewhere — by another jurisdiction, by another agency, or by another program. The Government of Alberta and the Government of Canada have access to every reference model named here.

9.1 Form Design

- All disability program application and compliance forms reviewed and rewritten to a Grade 6 reading level or below, validated by formal Flesch-Kincaid analysis and disability community user testing.
- Plain-language summaries placed at the top of every form, in the first paragraph, explaining what the form is for, what the recipient must do, and what happens if they do not.
- All deadlines, prerequisites, fees, and pathways disclosed in the same place as the requirement they relate to. A compliance notice that states a deadline must, in the same notice, explain the steps required to meet it.
- All forms available in alternative formats on request: large print, screen-reader-compatible digital format, audio recording, plain-language summary, ASL/LSQ video translation.
- Caseworker-assisted form completion available, free of charge, for any recipient who requests it or whose file shows a documented disability that may interfere with form completion.

9.2 Communication Design

- Restoration of named caseworker access for AISH recipients, with documented continuity and a clear escalation pathway.
- Compliance notices delivered in multiple channels (mail, email, phone call, optional text message) with confirmation of receipt before any deduction or hold can be applied.
- Mandatory accessibility check-in at any point in a recipient's file when a missed deadline is identified, before any sanction is applied.
- Free interpretation services available at all points of recipient contact, including ASL/LSQ, English-as-additional-language interpretation, and intellectual-disability-trained communication support.

9.3 Cost Removal

- Provincial coverage of all physician fees for AISH-required and ADAP-required form completion, billed directly to the program rather than reimbursed to the recipient.
- Federal coverage of physician fees for DTC certification at the point of service, eliminating the up-front cost barrier identified by the federal Disability Advisory Committee as a primary cause of the 76% abandonment rate.
- Free professional tax preparation for all AISH and ADAP recipients, available year-round, not only during CVITP volunteer clinic windows.
- Elimination of the dollar-for-dollar Alberta CDB clawback. Where the federal government is delivering a benefit explicitly designed to reduce disability poverty, the provincial absorption of the entire benefit nullifies the federal intent.

9.4 Compliance Framework Reform

- Replacement of the willful non-compliance frame with an accommodation-first frame. Where a recipient fails to meet a compliance requirement, the first administrative step is an accessibility review, not a benefit deduction.
- Restoration of independent appeal rights for AISH and ADAP eligibility decisions, removed by Bill 12 (December 2025).
- Establishment of a disability access ombudsperson within the Office of the Advocate for Persons with Disabilities, with statutory authority to investigate accessibility failures and require remediation.
- Annual public reporting of compliance failure rates, broken down by primary diagnosis category, to identify disproportionate impact and target remediation.

WHAT NONE OF THIS REQUIRES

None of the items above requires new technology, novel policy, or untested intervention. Each has been implemented in some form by another government, agency, or program. The barrier is not feasibility. The barrier is the absence of a political requirement to address documented inaccessibility. That requirement is what this report and the campaign behind it exist to create.

SECTION 10: WHAT IS BEING ASKED

10.1 To the Government of Alberta

- Conduct and publish a full accessibility audit of all AISH, ADAP, FSCD, and PDD application and compliance materials, including readability assessment against the Grade 6 plain-language standard.
- Implement an accommodation-first enforcement framework. No benefit reduction or hold may be applied for missed paperwork until an accessibility review has been completed and documented.
- Restore named-caseworker access for AISH and ADAP recipients, with documented continuity and a published escalation pathway.
- Cover, directly and at the point of service, all physician fees for AISH-required and ADAP-required form completion.
- End the dollar-for-dollar Alberta clawback of the federal Canada Disability Benefit. Alberta is the only province in Canada to do this. The clawback nullifies the federal poverty-reduction purpose of the CDB.
- Restore independent appeal rights for AISH and ADAP eligibility decisions, removed by Bill 12 (December 2025).
- Publish ADAP eligibility criteria with full transparency on the medical assessment standard, the personnel making decisions, the appeal pathway (or, in its absence, the alternative recourse mechanism), and the timeline for review.
- Establish a disability access ombudsperson within the Office of the Advocate for Persons with Disabilities, with statutory authority to investigate accessibility failures and require remediation.

10.2 To the Government of Canada

- Address the documented 76% abandonment rate of online Disability Tax Credit applications. The rate has been on the federal Disability Advisory Committee record since at least 2024. The remediation has not been implemented.
- Cover physician DTC certification fees at the point of service, federally, eliminating the up-front cost barrier identified as a primary cause of DTC abandonment.
- Legislate that the Canada Disability Benefit cannot be used as a provincial income offset. A non-binding call to provinces is not sufficient. Where a province eliminates the net federal benefit, the federal program is not delivering on its stated poverty-reduction goal.
- Conduct a full accessibility audit of CRA general inquiries access, including the identity verification protocol and the absence of a dedicated disability or accessibility line.
- Implement plain-language redesign of all federal disability benefit communications, validated by Flesch-Kincaid analysis and disability community user testing.
- Strengthen the Accessible Canada Act with mandatory remediation timelines for documented federal accessibility failures, including the DTC abandonment rate.

10.3 To the Disability Community

- Document every inaccessible interaction with government systems. Date, time, agency, what was required, what barrier was encountered, what happened. The documented record is the basis on which legal challenge becomes possible.

- File complaints — Alberta Human Rights Commission, Alberta Ombudsman, Office of the Advocate for Persons with Disabilities, Information and Privacy Commissioner. Complaints are free. They build the record. They create accountability pressure that does not exist when failures go unreported.
- Submit personal accounts to The Alberta Disability System Breakdown for inclusion in the campaign archive. Personal accounts grounded in documented experience are the strongest form of evidence in the public record.
- Share this report. Print it. Email it. Quote it. The pattern documented here is invisible only as long as the people experiencing it are isolated from each other. Visibility is the precondition for change.

THE CAMPAIGN'S POSITION

The Access Gap is not new. It has existed as long as the systems in this report have existed. What is new is the campaign's documentation of it, the community's organizing around it, and the political moment — the July 1, 2026 ADAP transition — that makes the cost of inaccessibility no longer absorbable by the population it has been imposed on. The conditions for change are present. What is required is the political pressure to convert them into change.

SOURCES AND REFERENCES

Government of Alberta — Primary Documents

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- Government of Alberta — AISH Direct Deposit Statement, January and February 2026. CDB compliance notice format and content.
- Government of Alberta — AISH Policy Manual. Non-taxable status of AISH benefits; reporting requirements; benefit reduction protocols.
- Government of Alberta — Alberta.ca, 'How to apply for AISH.' Explicit statement that physician fees for form completion are the recipient's responsibility.
- Government of Alberta — ADAP Program documentation, 2026. Transition timeline; current information on exemption categories.
- Government of Alberta — Bill 12 (December 9, 2025). Removal of independent appeal rights for AISH and ADAP eligibility decisions.
- Government of Alberta — Alberta Human Rights Act, R.S.A. 2000, c. A-25.5, section 4 (discrimination in services and accommodation duty).

Government of Canada — Primary Documents

- Canada Revenue Agency — Form T2201, Disability Tax Credit Certificate, and accompanying eligibility documentation. canada.ca.
- Government of Canada — Disability Advisory Committee Reports 2019, 2022, 2024. DTC application abandonment statistics; documented complexity barriers.
- Government of Canada — Canada Disability Benefit program documentation. Service Canada. Application process; eligibility prerequisites; payment schedule.
- Income Tax Act (Canada) — section 81(1)(h.2). Non-inclusion of social assistance payments in taxable income.
- Government of Canada — Accessible Canada Act, S.C. 2019, c. 10. Federal accessibility standard framework.
- Canadian Charter of Rights and Freedoms — section 15 (equality rights, including physical and mental disability).

International Instruments

- United Nations Convention on the Rights of Persons with Disabilities (UNCRPD), 2006. Ratified by Canada December 2010. Articles 9, 19, 21, 28 directly implicated.
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- Inclusion Alberta — Canada Disability Benefit Fact Sheet, July 2025.
- Maytree — Welfare Incomes Across Canada, 2024. AISH recipients \$6,004 below poverty line annually.

Companion Documents from The Alberta Disability System Breakdown Archive

- AISH Evidentiary Report Series, April 2026. Primary-source documentation of compliance record, File #1234567.
- Information Asymmetry Report — companion document to this report. Covers the information gap (what is not communicated). This report covers the access gap (what cannot be navigated).
- Healthcare Worker Shortage Report. 650,000 Albertans without a family doctor; 887 to 164 collapse in accepting practices.
- Financial Reality Report. AISH benefit levels and the documented poverty gap.
- Compound Failure Brief. Synthesis of AISH, ADAP, FSCD, PDD, healthcare access, and appeal rights as a single failing system.
- Charter Constitutional Analysis (Legal). Section 15 framework for substantive equality challenge.
- All companion documents available at albertadisabilitysystembreakdown.netlify.app — free to share, free to print, free to distribute.

ABOUT THIS REPORT

This report was compiled by The Alberta Disability System Breakdown — a community-led documentation campaign on the AISH-to-ADAP transition, FSCD/PDD waitlists, healthcare access, the federal Canada Disability Benefit clawback, and the structural inaccessibility of Alberta and federal disability systems. All figures sourced from Government of Alberta open data, Statistics Canada, peer-reviewed research, and primary-source government correspondence. Published May 2026. Free to share. Free to print. Free to distribute. albertadisabilitybreakdown@outlook.com | albertadisabilitysystembreakdown.netlify.app